

ASTHMA ACTION PLAN

This form expires 1 year after submission

Last, First, Name	Date of Birth	Date	 GREEN means Go! Use CONTROL medicine daily YELLOW means Caution! Add RESCUE medicine RED means EMERGENCY! Inhalers work better with spacers. Always use with a mask when prescribed.
Health Care Provider	Provider's Phone		
Emergency Contact	Parent's Phone	School	
Additional Emergency Contact	Contact Phone	Last Four Digits of SSN	

Asthma Triggers Identified (Things that make your asthma worse):

Circle what applies

Colds	Smoke (tobacco, incense)	Pollen
Animals	Strong odors	Mold/moisture
Dust	Pests (rodents, cockroaches)	Stress/emotions
Gastroesophageal reflux	Exercise	Seasons: Fall, Winter, Spring, Summer
Other: _____		

Date of last medical appointment: _____

Green Zone: Doing well-continue control medicines DAILY

You have **ALL** of these:

- Breathing is easy
- No cough or wheeze
- Can work and play
- Can sleep all night

Peak flow in this area: _____ to _____
(More than 80% of Personal Best)

Personal best peak flow: _____

Always rinse mouth after using your daily inhaled medicine. **Inhalers work better with spacers**

- No control medicines required.

■ _____ puff(s) (MDI) _____ times a day
Inhaled corticosteroid or inhaled corticosteroid/long-acting β -agonist

■ _____ nebulizer treatment(s) _____ times a day
Inhaled corticosteroid

■ _____, take _____ by mouth once daily at bedtime
Leukotriene antagonist

■ For asthma with exercise, ADD:
_____ puff(s) (MDI) 15 minutes before exercise
Fast-acting inhaled β -agonist

- For nasal/environmental allergy, ADD:

Yellow Zone: Caution! –Continue CONTROL Medicines and ADD RESCUE Medicines

When you have **ANY** of these:

- First sign of a cold
- Cough or mild wheeze
- Tight chest
- Problems sleeping, working, or playing
- Exposure to known trigger.

Peak flow in this area: _____ to _____
(50%- 80% of Personal Best)

■ _____ puff(s) MDI with spacer every _____ hours as needed
Fast-acting inhaled β -agonist

OR

■ _____ nebulizer treatment(s) every _____ hours as needed
Fast-acting inhaled β -agonist

- Other _____

Do not leave child alone, and if the child should feel better within 20-60 minutes of the quick-relief treatment. If the child is getting worse or is in the Yellow Zone for more than 24 hours, THEN follow the instructions in the RED ZONE and call the health care provider right away*

OR if you do feel better continue treatments every 4-6 hours as needed for 1-2 days.

Red Zone: EMERGENCY! – Continue CONTROL & RESCUE Medicines and GET HELP!

When you have **ANY** of these :

- Can't talk, eat, or walk well
- Medicine is not helping
- Breathing hard and fast
- Blue lips and fingernails
- Tired or lethargic
- Ribs show

Peak flow in this area: _____ to _____
(Less than 50% of Personal Best)

■ _____ puff(s) MDI with spacer every **15 minutes**, for **THREE** treatments
Fast-acting inhaled β -agonist

OR

■ _____ nebulizer treatment every **15 minutes**, for **THREE** treatments
Fast-acting inhaled β -agonist

Call your Healthcare Provider while giving the treatments.

- Other _____

**IF YOU CANNOT CONTACT YOUR HEALTHCARE PROVIDER:
Call 911 for an ambulance or go directly to the Emergency Department!**

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REQUIRED Healthcare Provider Signature:

_____, Date: _____

REQUIRED Parent/Guardian Signature:

_____, Date: _____

Follow up with primary care provider in 1 week or:

Health Care Provider Stamp below:

SCHOOL MEDICATION CONSENT AND PROVIDER ORDER FOR CHILDREN/YOUTH:
Possible side effects of rescue medicines (e.g., albuterol) include tachycardia, tremor, and nervousness.

Healthcare Provider Initials:

- _____ This student is not approved to self-medicate.
- _____ This student is capable and approved to self-administer the medicine(s) named above.

As the PARENT/GUARDIAN:

- _____ I hereby authorize a trained school employee, if available, to administer medication to the student.
- _____ I hereby authorize the student to possess and self-administer medication.
- _____ I hereby acknowledge that the District and its schools, employees and agents shall be immune from civil liability for acts or omissions under D.C. Law 17-107 except for criminal acts, intentional wrongdoing, gross negligence, or willful misconduct.

Criteria apply to all ages unless otherwise indicated	IMPAIRMENT						RISK	
	Daytime Symptoms 	Nighttime Awakenings <5 Years ≥5 years	Interference with normal activity	Short-acting beta-agonist use	FEV1 % predicted (n/a in age <5)	Exacerbations requiring oral systemic corticosteroids		
Classification of Asthma SEVERITY: TO DETERMINE INITIATION OF LONG-TERM CONTROL THERAPY Consider severity and interval since last exacerbation when assessing risk.								Step
Severe Persistent	Throughout the day	>1x/week	Often 7x/week	Extremely limited	Several x/ day	<60%	<5: ≥2 in 6 months OR ≥4 wheezing episodes in 1 year lasting >1 day AND risk factors for persistent asthma 5-11: Step 3 Medium-dose ICS option or Step 4 12-adult: Step 4 or 5 <i>All ages: Consider short course OCS</i>	
Moderate Persistent	Daily	3-4x/ month	>1x/week but not nightly	Some	Daily	60-80%	<5: Step 3 5-11: Step 3 Medium-dose ICS option or Step 4 12-adult: Step 4 or 5 <i>All ages: Consider short course OCS</i>	
Mild Persistent	>2 days/week but not daily	1-2x/ month	3-4x/month	Minor	>2 days/ week but not daily	>80%	5-adult: ≥2/year	Step 2
Intermittent	≤2 days/week	0	≤2x/month	None	≤2 days/week	>80%	0-1/year	Step 1

Classification of Asthma CONTROL: TO DETERMINE ADJUSTMENTS TO CURRENT CONTROL MEDICATIONS Consider severity and interval since last exacerbation and possible medication side effects when assessing risk. <12 years 12-adult								Action: In children <5, consider alternate diagnosis or adjusting therapy if no benefit seen in 4-6 weeks.
Very Poorly Controlled	Throughout the day	≥2x/week	≥4x/week	Extremely limited	Several times/day	<60%	<5: >3/year 5-adult: ≥2/year	Step up 1-2 steps. Consider short course OCS. Reevaluate in 2 weeks. For side effects, consider alternate treatment.
Not Well Controlled	>2 days/week	≥2x/ month	1-3x/week	Some	>2 days/week	60-80%	<5: 2-3/year 5-adult: ≥2/year	Step up at least 1 step. Reevaluate in 2-6 weeks. For side effects, consider alternate treatment.
Well Controlled	≤2 days/week	≤1x/month	≤2x/month	None	≤2 days/week	>80%	0-1/year	Maintain current treatment. Follow-up every 1-6 months. Consider step down if well controlled for at least 3 months.

Daily Doses of common inhaled corticosteroids	Fluticasone			Budesonide			Beclomethasone			Fluticasone/ Salmeterol	Budesonide/ Formoterol
	MDI (mcg)			Respules (mcg)			MDI (mcg)			DPI	MDI
	Low	Medium	High	Low	Medium	High	Low	Medium	High		
<5 years	176	>176-352	>352	0.25-0.5	>0.5-1	>1	n/a	n/a	n/a	n/a	n/a
5-11 years	88-176	>176-352	>352	.5	1	2	80-160	>160-320	>320	100/50 mcg 1 inhalation BID	80 mcg/4.5 mcg 2 puffs BID
12 years-adult	88-264	>264-440	>440	n/a	n/a	n/a	80-240	>240-480	>480	Dose depends on patient	Dose depends on patient

Abbreviations:
SABA: Short-acting beta-agonist
LABA: Long-acting beta-agonist
LTRA: Leukotriene-receptor antagonist
ICS: Inhaled corticosteroids
LD-ICS: Low-dose ICS
MD-ICS: Medium-dose ICS
HD-ICS: High-dose ICS
OCS: Oral corticosteroids
CRM: Cromolyn
NME: Nedocromil
THE: Theophylline
MLK: Montelukast
ALT: Alternative

Abbreviations: SABA: Short-acting beta-agonist LABA: Long-acting beta-agonist LTRA: Leukotriene-receptor antagonist ICS: Inhaled corticosteroids LD-ICS: Low-dose ICS MD-ICS: Medium-dose ICS HD-ICS: High-dose ICS OCS: Oral corticosteroids CRM: Cromolyn NCM: Nedocromil THE: Theophylline MLK: Montelukast ALT: Alternative					
Step 1 <u>Preferred</u> SABA prn	Step 2 <u>Preferred</u> LD-ICS <u>Alternative</u> <5: CRM or MLK 5-adult: CRM, LTRA, NCM, or THE	Step 3 <u>Preferred</u> <5: MD-ICS 5-11: <i>EITHER</i> LD-ICS plus LABA, LTRA or THE <i>OR</i> MD-ICS 12-adult: LD-ICS plus LABA <i>OR</i> MD-ICS <u>Alternative</u> 12-adult: LD-ICS plus either LTRA, THE or Zileuton	Step 4 <u>Preferred</u> <5: Medium-dose ICS plus either LABA or MLK 5-adult: MD-ICS plus LABA <u>Alternative</u> 5-11: MD-ICS plus either LTRA or THE 12-adult: MD-ICS plus either LTRA, THE or Zileuton	Step 5 <u>Preferred</u> <5: HD-ICS <i>plus either</i> LABA or MLK 5-11: HD-ICS <i>plus</i> LABA 12-adult: High-dose ICS <i>plus</i> LABA <i>AND</i> consider Omalizumab for patients who have allergies <u>Alternative</u> 5-11: HD-ICS <i>plus either</i> LTRA or THE	Step 6 <u>Preferred</u> <5: HD-ICS <i>plus either</i> LABA or MLK <i>plus</i> OCS 5-11: HD-ICS <i>plus</i> LABA <i>plus</i> OCS 12-adult: HD-ICS <i>plus</i> LABA <i>plus</i> OCS <i>AND</i> consider Omalizumab for patients who have allergies <u>Alternative</u> 5-11: HD-ICS <i>plus either</i> LTRA or THE <i>plus</i> OCS

← Step down if possible (asthma well-controlled at least 3 months)/Step up if needed (check adherence, technique, environment, co-morbidities) →